



GLOBAL HEALTH

Fight against neonatal mortality is losing steam

More than 60 countries are off track to meet U.N. target for 2030—even though solutions exist

ABDULLAHI TSANNI, in Nairobi, Kenya

In 2015, as part of its Sustainable Development Goals (SDGs), the United Nations set an ambitious target for the world: Bring down neonatal deaths to 12 per 1000 live births or fewer by 2030. But with only 4 years to go, more than 60 countries are well off track. In Kenya, for example, neonatal mortality, defined as a baby dying within the first month of life, has declined by just one death per 1000 live births—from 22 to 21—since 2014.

“We’re failing newborns,” says Joy Lawn, a neonatal clinician and professor at the London School of Hygiene & Tropical Medicine (LSHTM).

Last week, policymakers, researchers, and advocates gathered here at the International Maternal Newborn Health Conference to discuss why progress has slowed down, especially in Africa, which has about 1.1 million newborn deaths annually, and how to get it back on track. Fixes as simple as better training and record-keeping can help, studies presented at the meeting show. But money and political will are often lacking. The dramatic cuts in global health aid, including the dismantling of the U.S. Agency for International Development, may undo progress made over the past 2 decades.

A U.N. report on child mortality, released on 17 March, estimates there are still 2.3 million neonatal deaths every year—a toll larger than that of HIV and many other high-profile

diseases. Prematurity accounts for about 18% of deaths, followed by birth asphyxia and trauma, pneumonia, malaria, and diarrhea. “We know what is killing these babies,” says William Macharia, a professor of pediatrics and child health at Aga Khan University. “We are not lacking solutions.”

Most women in Africa give birth in health facilities—a safer setting than at home. But studies presented at the meeting showed their care is often inadequate. One tracked power outages in neonatal units in 60 hospitals across four countries—Malawi, Tanzania, Nigeria, and Kenya—for 18 months. On average, hospitals had an outage on nearly 200 days, disrupting incubators, oxygen supply, and monitoring equipment. A shortage of specialized nurses is another key problem. Tanzania alone faces a shortfall of more than 2000 nurses for neonatal care.

In Kenya, an analysis of data on more than 130,000 newborns showed babies that were transferred to another hospital were three times more likely to die than those who received treatment in the hospital they were born in. Part of the reason is that the sickest babies are more likely to be referred, but the finding may also reflect poor care at the original facility, unsafe transportation, and admission delays.

An extensive package of interventions can help improve things, however. In 2019, 23 organizations,

most of them in Africa, formed a partnership called Newborn Essential Solutions and Technologies, or NEST360, that aims to improve care at 130 hospitals in the five countries with the highest neonatal mortality: Kenya, Malawi, Nigeria, Tanzania, and Ethiopia. It helps provide low-cost equipment, such as incubators and thermal mattresses to keep babies warm, and machines for breathing support and jaundice management.

NEST360 also helps train doctors and engineers to keep equipment working and teach staff how to use it. Respiratory support, for example, can be “lifesaving,” says Lawn, who coordinates NEST360’s economic and impact group, but “safe use requires careful monitoring of oxygen levels to avoid complications such as eye damage.” NEST360 also promotes nontechnological interventions, such as “kangaroo mother care,” or skin-to-skin contact between infants and their parents, which has been shown to increase survival.

Collecting data—on information such as facility of birth, birth weight, care received, and clinical diagnosis on admission—is key, NEST360 researchers have found. Good records can help hospital workers make better decisions, show what’s working and what’s failing, and guide government plans, Lawn says.

Early analyses from 65 participating hospitals show marked declines

Mothers and their babies in a maternity ward in Narok, Kenya, a country where neonatal mortality remains high.

in newborn mortality, according to an interim report. Some of the biggest gains are among the most vulnerable infants, particularly preterm babies who struggle to breathe, Lawn says. Malawi stands out: It has reduced neonatal deaths in hospitals by 23% between 2019 and 2025 and is one of the few countries in Africa that may reach the SDG target by 2030. “There’s still encouraging news for Africa,” Lawn says. “But progress is not fast enough.”

Some South Asian countries with high neonatal mortality have made strides as well. Findings from a series of papers set to be published later in *The Lancet* showed Pakistan has made steady progress between 1990 and 2024, although the country is still not on track to meet the SDG target. India, Nepal, and Bangladesh have all made significant gains as well.

But a lack of funding is imperiling that progress. Newborn health has long been underfunded compared with maternal

and reproductive health, says Alice Tarus, a health economist and Ph.D. student at LSHTM who researches the cost of newborn care in African countries: “The assumption has been when you fund maternal health, by extension, you’ve already covered the baby, which is not true,” she says.

And things got a lot worse last year. A December 2025 report from the global health nonprofit PATH estimated that donor aid for maternal, newborn, and child health fell by 49% in 2025, from \$1.66 billion to about \$850 million. The report forecasts up to 8 million additional child deaths and more than 1 million maternal deaths by 2040 if the money isn’t replaced. Tarus says African governments must increase domestic budgets for newborn care, in part because it’s in their own interest: “Investing in newborn care is both a moral and economic imperative.” □

Abdullahi Tsanni is a science journalist.

TRUMP ADMINISTRATION

Agency for patient care research is in dire straits

The Agency for Healthcare Research and Quality has grant money, but nobody to hand it out **JOCELYN KAISER**

Its staff gutted by the White House last year, a small federal agency with a focus on studying patient care inched closer to death last month when it canceled all open grant competitions. Yet Congress has approved \$345 million in funding this year for the Agency for Healthcare Research and Quality (AHRQ), and supporters are pushing Trump officials to revive it.

The 37-year-old agency normally awards about \$200 million a year in research grants, but has issued no grants in the fiscal year that began on 1 October 2025, even for ongoing projects. And after nearly 20 calls for proposals expired in March, the agency’s website now says it has “no current open opportunities.” “In practice, the agency has largely stopped functioning,” says Aaron Carroll, president and CEO of AcademyHealth, a research advocacy group.

The plight of AHRQ is “devastating” for researchers who study health services, including whether treatments are helping patients in the real world, says infectious disease researcher Deverick Anderson of Duke University, who has AHRQ support. “I have significant worry that entire portfolios of highly impactful research will no longer have a funding home.”

A spokesperson for the Department of Health and Human Services (HHS), which oversees AHRQ, said the agency “is fully operational and focusing on existing priorities” and “expects to issue noncompeting continuation grants”—the annual installments for previously approved grants.



IN FOCUS

This bird literally sucks

Butterflies suck—nectar. Walruses suck—clams. And most fishes suck everything they eat. But birds? Scientists had seen only a few examples, such as in pigeons, of drinking by suction. Last week in *Current Biology*, researchers reported sunbirds use suction from their tongue to sip nectar, the first example of any vertebrate drinking through suction generated by the tongue alone, rather than by changing the shape of its mouth. The discovery came about as biologist David Cuban, then a Ph.D. student at the University of Washington, filmed the feeding behavior of seven sunbird species in South Africa and Indonesia. The animals have a V-shaped trough running along the top of their tongue that, when pressed against the beak, creates suction when the tongue is pulled down. Sunbirds coevolved with flowers, and Cuban thinks their nectar-sucking tongue shaped that history. —Erik Stokstad



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